

**BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL
COMMISSION, GHAZIABAD (U.P.)**

CONSUMER COMPLAINT NO. _____ OF 2026

(Complaint under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Mr. Vikas Gandhi

...Complainant

VERSUS

ACKO General Insurance Limited

...Opposite Party

LIST OF ANNEXURES

Annexure	Document
C-1 (colly.)	Policy Schedule, Policy No. ASHP202400001607 (with "Additional conditions" and Annexure – Specific illness list); renewal Policy Schedule No. ASHP202400001607R1 (04.11.2025–03.11.2026)
C-2	Proposal Form (recording "No medical conditions"; "PED declared — Obesity")
C-3	Premium payment records / policy active status
C-4	Discharge Summary, RS Maitri Health Care, Ghaziabad (20–21.06.2025)
C-5	Hospital bills with break-up of charges (₹37,358/-)
C-6	Claim registration and Complainant's representation dated 24.06.2025
C-7	GRO's rejection email dated 25.06.2025
C-8	Repudiation email dated 26.06.2025
C-9	Histopathology Report, Max Lab, dated 30.06.2025 (No. V-6630/25)
C-10 (colly.)	Subsequent appeals/grievance correspondence with the Opposite Party
C-11	Award of Insurance Ombudsman, Noida, No. IO/NOI/A/HI/0061/2026-2027 dated 02.07.2026, with forwarding letter (Annex-XIII-A)
C-12	Medical Certificate dated 12.07.2026 of the treating super-specialist, Dr. Vaibhav Saxena, M.S. (BHU), M.Ch. Urology (AIIMS, New Delhi), Senior Director – Urology, Uro-Oncology, Robotic Surgery & Renal Transplant, Yashoda Super Speciality Hospital, Kaushambi; U.P. Medical Council Reg. No. 47425
C-13 (colly.)	Medical literature on BXO / Lichen Sclerosus as a premalignant condition: (C-13A) EAU Guidelines on Penile Cancer 2022 (highlighted pages); (C-13B) Bunker CB, Shim TN, “Male Genital Lichen Sclerosus”, Indian J Dermatol (review) — see note below
C-14	Copy of Aadhaar / address proof of Complainant

Note on Annexure C-13 (Medical Literature — on record, colly.)

Two authoritative medical documents are annexed as C-13 (colly.), establishing that BXO / genital Lichen Sclerosus is a recognised premalignant condition associated with penile squamous cell carcinoma, and that phimosis (the Complainant's recorded diagnosis) is itself strongly associated with invasive penile cancer:

1. C-13A — European Association of Urology (EAU) Guidelines on Penile Cancer (2022): Table 1 records phimosis as a risk factor with an odds ratio of 11–16 versus no phimosis and states that “Phimosis is strongly associated with invasive penile cancer ... due to associated chronic infection”; and Table 4 expressly classifies “Lichen sclerosus” among “Premalignant penile lesions (precursor lesions)” — the very biopsy diagnosis in the present case.
2. C-13B — Bunker CB, Shim TN, “Male Genital Lichen Sclerosus” (Review Article, Indian Journal of Dermatology): records that the feared complication of male genital lichen sclerosus is squamous cell carcinoma of the penis; that “the association of GLSc and SCC is widely recognized and the risk has

been estimated at between 2% and 12.5%” (bracketing the “about 6%” figure noted at the Ombudsman hearing); and that lichen sclerosus is one of the two recognised pathways for the development of penile cancer. This review also cites both Nasca et al. (J Am Acad Dermatol 1999) and Depasquale et al. (BJU Int 2000), the primary sources on the point.

(Relevant highlighted pages are annexed; the treating specialist may also be requested to reference/append this literature to his certificate at Annexure C-12.)

Place: Ghaziabad

Date: ____ July 2026

COMPLAINANT
(Vikas Gandhi)